

NAME :- SAROJ AGRAWAL	AGE :- F/63 YRS
REF. BY :- DR. KAUSHAL B. PATEL Sir (MD, DM Medical Oncology)	DATE :- 22/06/2026

18 F-FDG WHOLE BODY DIGITAL PET-CT STUDY

Protocol:

5.38 mCi 18 F-FDG was injected after 5-6 hrs of fasting state. Scan was performed on **AI ASSISTED 160 SLICE DIGITAL PET-CT SCAN** was performed 60 min after the injection on. Semi-quantitative analysis of FDG uptake was performed by calculating SUV value based on body weight. A separate breath hold CT was also acquired. Extent of the scan: Vertex to mid-thigh.

Referral Reasons: Known case of Ca. endometrium with lung metastasis

11/02/2026 - NT-proBNP – 1738 (high), CPK-MB – 34.67 (high), Troponin-I - negative
12/02/2026 – No organism seen. Negative AFB. MTB not detected.
CA-125 (14/02/2026) – 635.50
Cytopathology (14/02/2026) - Pleural fluid - Few clusters of atypical cells - suggestive of carcinoma with large cell morphology.
Post immuno and chemotherapy (LD 15/06/2026)
C/o breathing difficulty on and off.
For follow-up evaluation.

Findings:

BRAIN:

Physiological FDG distribution is noted throughout the cerebral parenchyma without focal hypermetabolic lesion.

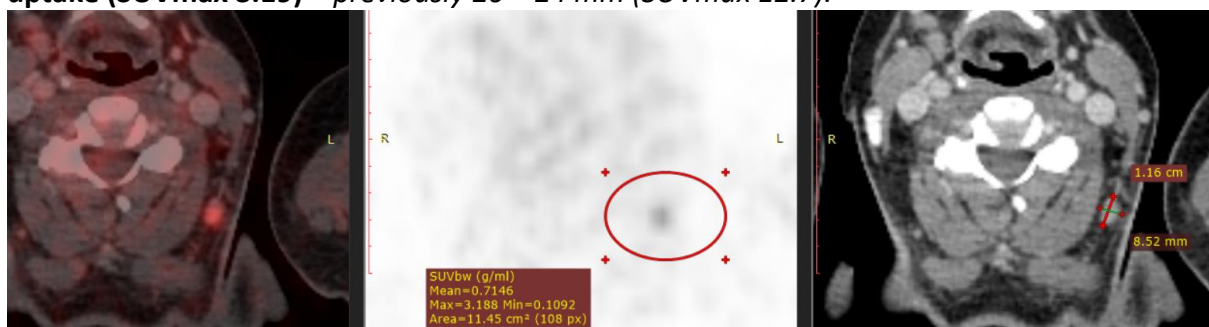
No evidence of intracranial metastatic disease on the present study.

HEAD & NECK

Physiological FDG uptake is noted in the visualized pharyngeal lymphoid tissue.

Previously noted metabolically active necrotic left cervical nodal disease demonstrates marked interval regression.

Residual left level V cervical lymph node measures approx. 12 × 8 mm with low-grade FDG uptake (SUVmax 3.19) – previously 20 × 24 mm (SUVmax 12.7).



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Mild diffuse symmetrical FDG uptake is noted involving both thyroid lobes without focal CT abnormality, likely inflammatory/reactive in nature.

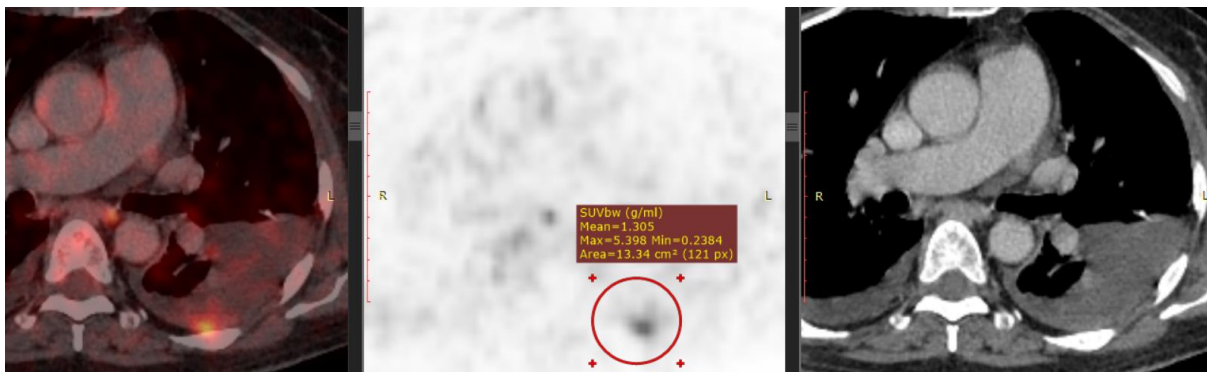
No new metabolically active cervical lymphadenopathy.

CHEST

Interval significant reduction in bilateral pleural disease – now mild residual bilateral pleural effusions are noted, greater on the left side.



Previously noted right pleural hypermetabolic thickening demonstrates metabolic regression with residual low-grade uptake, maximum SUVmax 5.4 - previously SUVmax 5.6.



No focal FDG avid pulmonary parenchymal lesion is identified.

Mild bi-basal compressive atelectatic changes are noted.

Previously described mediastinal nodal disease has markedly regressed. Tiny residual prevascular lymph node measuring approx. 8 × 7 mm demonstrates mild FDG uptake (SUVmax 4.93). Tiny right upper paratracheal nodes show low-grade uptake.

No significant FDG avid hilar lymphadenopathy.



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Bilateral axillary lymph nodes are predominantly small, oblong in morphology and demonstrate only low-grade tracer uptake, likely stable.

No e/o pericardial effusion.

No evidence of residual pulmonary arterial thrombus on the CT component of the study.

ABDOMEN AND PELVIS

Operated bed:

Post hysterectomy status is noted.

No abnormal FDG avid lesion is identified at the operative bed.

Lymph node, omentum and peritoneum:

Significant interval regression of previously noted omental and peritoneal metastatic disease. Residual tiny metabolically active peritoneal/omental nodule is seen measuring approx. 8.5 × 6.8 mm (SUVmax 7.12). Previously seen extensive supramesocolic omental nodularity is no longer measurable.

Mild residual ascites is noted.

Calcified lymph nodes are seen along bilateral common iliac chains without significant metabolic activity.

Previously noted abdominal nodal disease has substantially regressed without significant residual FDG avid lymphadenopathy.

Left inguinal lymph node now measures approx. 17 × 10 mm with mild FDG uptake (SUVmax 2.85) – previously 23 × 14 mm (SUVmax 6.4).



Multiple simple hepatic cysts are again noted, largest within segment VI, 3.2cm sized.

Small calcified granuloma is seen within segment III of the liver.

No FDG avid hepatic metastasis.

Spleen, pancreas, adrenal glands and kidneys show no focal hypermetabolic lesion.

Mild subcutaneous edema is noted involving the lower anterior pelvic wall.

MUSCULOSKELETAL:

No FDG avid osseous metastatic lesion. No suspicious metabolically active skeletal lesion.

Mild degenerative spondylotic changes are noted.

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CONCLUSION:

As compared to prior PET-CT (reports & limited DICOM data) dated 23.02.2026, the present PET-CT demonstrates:

1. Marked interval regression of previously noted left cervical metastatic lymphadenopathy, with residual small left level V node measuring 12×8 mm (SUV_{max} 3.19), previously 20×24 mm (SUV_{max} 12.7).
2. Significant regression of mediastinal, axillary, infraclavicular, abdominal and inguinofemoral nodal disease with only small residual mildly FDG avid nodes.
3. Significant regression of previously noted peritoneal carcinomatosis and omental metastatic deposits, with only tiny residual metabolically active peritoneal nodule measuring 8.5×6.8 mm (SUV_{max} 7.12).
4. Interval reduction in bilateral malignant pleural disease with persistent mild bilateral pleural effusions and mild residual pleural FDG uptake ($maximum\ SUV_{max}$ 5.4).
5. Mild residual ascites.

RECIST 1.1 Assessment

Target lesions:

- Left level V cervical node: 20 mm $\rightarrow$ 8 mm short axis
- Left inguinal node: 14 mm $\rightarrow$ 10 mm short axis

Sum of target lesion diameters:

- Previous: 34 mm
- Current: 18 mm

Non-target lesions:

- Bilateral pleural metastatic disease: improved; mild residual pleural effusions/thickening persist.
- Omental/peritoneal carcinomatosis: near-complete regression with tiny residual pelvic peritoneal measurable deposit as described.
- Mediastinal nodal disease & abdominal nodal disease: significant regression without appreciable residual FDG avidity.

PERCENTAGE CHANGE: -47.1%

RECIST 1.1 OVERALL RESPONSE: PARTIAL RESPONSE (PR)

Overall findings indicate a significant favourable treatment response with marked reduction in nodal, pleural and peritoneal metastatic disease burden, with residual metabolically active disease as described.



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